

Toddler Biting at Daycare: A Parent's Guide to the Crisis and the Path Forward

Introduction

The phone call comes at 11:47 AM. It's the daycare director. Your son bit another child. This is the third time this month.

You leave work early — again. The director tells you, calmly but firmly, that your son cannot return until you have a "behavior plan." You don't know what that means. You don't know where to start. Your son is 22 months old and has maybe 15 words. You love him more than anything and you are drowning.

This guide is for you.

It covers what biting actually means at this age, why it isn't your fault, how to identify what is triggering your child, what to do in the moment, how to work with your daycare, and how to teach your toddler a different way to communicate. You will finish this guide with a concrete plan — not a guarantee, but a path.

Page 1: What Biting Actually Means at This Age

Biting in toddlers between 18 and 30 months is almost never malicious. It is not a character flaw. It is not evidence that your child has a behavioral disorder. It is a behavior — with identifiable causes and identifiable solutions.

The research is consistent on this point:

- The American Speech-Language-Hearing Association (ASHA) states that biting is frequently a "communication strategy for toddlers who have not yet developed adequate verbal skills." Your child is not trying to hurt anyone. Your child is trying to tell you something they do not have words for yet.
- Zero to Three, the leading organization on early childhood development, describes toddler biting as an expression of frustration, overstimulation, fear, or the need for control — all emotions that exceed what a pre-verbal or early-verbal child can articulate.
- The American Academy of Pediatrics notes that this behavior peaks during the 18–24 month window precisely because children at this age have strong emotions and opinions but limited language to express them.

Your toddler is not choosing to bite. Your toddler is biting because something in their world — inside their body, in their environment, or in a social situation — has exceeded their capacity to cope with words, and biting is the next option.

Understanding this is not about excusing the behavior. It is about finding the right fix.

Page 2: The Five Bite Types

Not all biting has the same cause. Before you can fix it, you need to know what is triggering it. Here are the five types most commonly seen in the daycare setting.

1. Frustration Biting

This is the most common type. It occurs when a toddler wants something, cannot have it, and has no words to protest. The trigger might be another child taking a toy, being told "no," or wanting attention from a caregiver who is busy with another child.

Signs: biting during transitions, after being told no, when another child has something they want.

2. Sensory Biting

Some children bite because they crave sensory input. Their nervous systems need more sensory stimulation than they are getting, and biting provides it. This is different from frustration — these children may bite even when not upset.

Signs: biting when overstimulated OR under-stimulated, biting objects (crib rail, toys), chewing on non-food items beyond the typical mouthing of infancy.

3. Impulse Biting

The prefrontal cortex — the part of the brain responsible for stopping an action before it happens — does not fully develop until ages 3 to 4. A toddler's brain physically cannot always apply the brake in the moment between impulse and action.

Signs: biting appears without clear warning, child seems surprised by their own behavior afterward, biting in situations of high excitement or frustration equally.

4. Attention Biting

Some toddlers discover that biting produces a strong reaction — from adults, from other children, from anyone. Any reaction, including an upset reaction, can reinforce the behavior because attention is attention.

Signs: biting when a caregiver has been looking away, biting to provoke a response, escalation when reaction is high-energy.

5. Defensive Biting

A child who has been bitten before, or who feels threatened or cornered, may bite preemptively. This is a fear response — the child's assessment that the situation is dangerous overrides social learning.

Signs: biting when approached by other children, biting after being in close contact with a child who is physically active, biting in crowded spaces.

Page 3: Medical Triggers to Rule Out First

Before you build a behavior plan, make sure there is not a physical cause driving the biting. Medical triggers are frequently overlooked because they are not behavioral — but they lower frustration thresholds significantly.

Teething

Molars and canines erupt between 18 and 24 months. This causes genuine, significant pain. A child in teething pain has a much lower capacity to tolerate frustration. Teething pain can last weeks per tooth and some children have multiple teeth coming in simultaneously.

Look for: drooling, gum swelling, nighttime waking that has newly started, chewing on hard objects, decreased appetite.

Ear Infections

Ear infections cause pain that children cannot articulate. An undiagnosed ear infection can make a previously calm child irritable and prone to lashing out. They are common in this age group and frequently missed because toddlers cannot say "my ear hurts."

Look for: pulling at ears, fluid draining from ears, recent cold followed by increased irritability, disturbed sleep, fever.

Constipation

This is widely under-recognized. Constipation causes abdominal discomfort that makes children irritable and less able to cope with minor frustrations. It is extremely

common in toddlers transitioning from breastmilk or formula to solid foods, and can be worsened by diets low in fiber and water.

Look for: straining, hard pellet stools, withholding behavior, refusing to sit on the potty, abdominal bloating.

Vision or Hearing Changes

A child who cannot see well or hear well may become more frustrated because their world does not make sense. Recent vision or hearing changes are easy to miss in a toddler who cannot describe what they are experiencing.

When to see your pediatrician: If you notice sudden changes in behavior, sleep disruption, or increased frustration without a clear social trigger, bring up the possibility of an ear infection, constipation, or other physical cause at your next visit. Rule these out before assuming it is purely behavioral.

Page 4: The Biting Intervention Protocol

When a bite happens, your response in the first 60 seconds matters most. Here is a step-by-step protocol you can share with your daycare and use at home.

Step 1: Immediate Safety (Seconds 0–10)

Your first priority is the bitten child. Move to the child who was bitten. Stay calm — your emotional state is being watched and absorbed by every child in the room.

Script: "I see [child's name]. [Your child]'s teeth hurt [child's name]. Let me help."

Step 2: Brief, Neutral Response to Your Child (Seconds 10–30)

Look at your child. Say clearly and without anger:

Script: "Teeth are not for biting. Biting hurts [child's name]."

Do not lecture. Do not shame. Do not put your child in another person's face to say sorry. This adds stress without teaching anything.

Step 3: Redirect and Repair (Seconds 30–60)

Give the bitten child positive attention. This models empathy and does not reward the biter — it simply shows that kind hands and words are how we treat people.

Do not say "Say sorry." A toddler cannot internalize apology in the moment. Instead: Model it yourself. "I am going to help [child's name]. I'm sorry their arm hurts."

Step 4: After the Dust Settles — With Your Own Child

Once the immediate moment has passed and both children have moved on, go to your child. At home or in a quiet moment with your child at daycare:

Script: "When you were upset about [the toy / being told no / the noise], your body wanted to bite. Biting hurts. Next time, you can say 'help' or come find me."

This is teaching, not punishment. Punishment does not teach a new skill. Teaching gives your child an alternative.

What NOT to Do

- Do not bite your child back. This teaches that biting is an acceptable way to communicate.
 - Do not put your child in time-out without explanation. Isolation without teaching does not work.
 - Do not say "bad boy" or "that's naughty." This creates shame without providing a solution.
 - Do not ignore the bitten child. Children watch how adults respond to injury.
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Page 5: Working With Your Daycare

You are not alone in this. Your daycare has seen biting before, and you have rights and options.

What to Ask Your Daycare Provider

1. "Can you describe exactly what happened — where were the children, what led up to the bite, how did the children respond?" — You need specifics to identify the bite type and trigger.
2. "What does your biting protocol look like?" — Many daycares have an existing framework. Ask to see it in writing.
3. "Can we create a written behavior plan together?" — Request this. It forces everyone to get specific and creates accountability.
4. "How will you communicate incidents to me and what documentation do you keep?" — You want incident reports in writing, not just verbal updates.
5. "What support does your program offer for children with speech delays?" — If your child is late on words, this matters.

Your Rights as a Parent

In most US states, a daycare cannot expel your child solely for a disability-related behavior without first engaging in an interactive process to provide support. If your child has a diagnosed speech delay or other developmental difference, you may have additional protections under the Americans with Disabilities Act (ADA).

Even without a formal diagnosis, you have the right to:

- Request a meeting to discuss a behavior support plan
- Ask for incident reports in writing
- Request early intervention screening through your state's Part C program (for children under 3) or your local school district (for children 3 and up)
- Request that the daycare document the triggers and patterns

The Incident Log

Ask your daycare to keep a log for your child. Each entry should include: date, time, location in the room, what was happening before the bite, what the child who was bitten was doing, what the response was, and who responded.

Share this log with your pediatrician. It helps identify patterns that are not visible in a single visit.

Page 6: The Communication Replacement System

The single most effective long-term intervention for frustration-based biting is teaching your child an alternative way to communicate.

Research from the American Speech-Language-Hearing Association confirms that teaching functional communication alternatives — gestures, signs, or words that serve the same purpose as the biting — reduces frustration-based aggressive behaviors significantly and often within two to four weeks.

This is called Functional Communication Training (FCT) in the speech pathology literature. You do not need a speech therapist to start it — you can begin today.

The Five Signals Every Toddler Can Learn

Teach these five signals in order. Use them yourself — as a parent — before your child needs to use them. Model the behavior you want to see.

1. **"Help"** — Hand reaching up, palm up, like asking to be picked up. Use this when your child needs assistance with something. Model it yourself when you need help with something in front of your child.
2. **"More"** — Fingertips together, hands coming apart. Use this when your child wants more of something — more food, more play, more time. Model it during meals and play.
3. **"Stop"** — Hand flat, palm out, pushed gently forward. Use this when your child wants someone to stop an action — stop taking the toy, stop a touch, stop a sound.
4. **"Mine"** — Hand to chest, tapping. Use this when your child wants to claim something. Model it with your own items.
5. **"All done"** — Both hands, palms up, shrug motion. Use this when your child is finished with an activity. Model it during transitions.

The Two-Minute Daily Practice

Pick one signal — "help" is usually the most useful and the most motivating — and practice it twice a day for two minutes during a low-stress activity.

During snack: "Do you want more grapes? Show me more." Pause. Wait. If your child does not sign, sign for them: "More — like this." Then give them more grapes. Repeat three to five times.

During play: "Do you need help with that tower? Show me help." Pause. Wait. If your child does not sign, sign for them, then help.

The repetition creates the neural pathway. Your child will start signing before they start saying the word, and the signing will reduce the frustration that causes biting.

When to Request a Speech Evaluation

If your child has fewer than 50 words at 18 months, or fewer than 200 words with no two-word combinations at 24 months, ask your pediatrician for a referral to a speech-language pathologist. Many insurance plans cover this. Early intervention services (Part C IDEA) provide free developmental screening and, if eligible, free speech and occupational therapy for children under 3 in the United States.

You do not need a diagnosis to request an evaluation.

Page 7: When to Seek Professional Help

This guide covers what you can do at home and in partnership with your daycare. There are situations where professional support makes a significant difference.

Speech-Language Pathology

If your child is using biting as their primary communication tool and has limited verbal output, a speech-language pathologist can create a targeted communication plan. This is not a last resort — it is an evidence-based intervention that often works faster than behavioral approaches alone because it addresses the root cause.

Early Childhood Mental Health Consultation

Some communities have early childhood mental health consultants who work specifically with toddlers and daycare settings. They observe the classroom, identify triggers, and help staff adapt the environment. Ask your daycare if they work with a consultant or if they can bring one in.

Behavioral Consultation

If biting incidents persist beyond four to six weeks of consistent home and daycare intervention, a board-certified behavior analyst (BCBA) with early childhood experience can create a formal behavior intervention plan. This is especially relevant if biting is causing injury or threatening your child's childcare placement.

Your Pediatrician

Bring the incident log. Ask specifically about: - Physical causes: ear infection, constipation, vision, hearing - Developmental screening: ask for the Ages and Stages Questionnaire (ASQ) or M-CHAT if you have concerns about broader development - Referral options for speech and behavioral support

Page 8: Quick-Reference Checklist

Use this page as your quick-reference during high-stress moments.

Bite Type Identification - [] Frustration Biting — no words for what they want or don't want - [] Sensory Biting — seeking or avoiding sensory input, not emotionally triggered - [] Impulse Biting — no warning, surprised by own behavior - [] Attention Biting — responds to adult reaction, positive or negative - [] Defensive Biting — fear-based, after being startled or cornered

60-Second Response Script 1. Move to bitten child. Stay calm. 2. To your child: "Teeth are not for biting. Biting hurts [name]." 3. Give positive attention to the bitten child. Model empathy. 4. Later, teach: "You can say help / show me."

Five Signals to Teach 1. Help — reaching hands up, palm up 2. More — fingertips together, pulling apart 3. Stop — flat hand, palm out, gentle push 4. Mine — hand to chest, tapping 5. All done — both hands, palms up, shrug

Daycare Conversation Starters 1. "Can you describe exactly what happened?" 2. "Can we create a written behavior plan together?" 3. "What documentation do you keep?" 4. "What support can your program offer?" 5. "Will you log incidents so we can spot patterns?"

Red Flags That Need a Pediatrician Visit - Biting suddenly increases with no behavioral trigger - Biting at home is as frequent as at daycare (not just a daycare-context behavior) - Your child has fewer than 50 words at 18 months - Biting causes broken skin, bruising, or injury to the face or neck - Your child seems disconnected or distressed in ways that don't fit typical toddler behavior

Page 9: Next Steps — Your Bonus

This guide comes with two downloadable tools to help you take action immediately.

Tool 1: Bite Type Identification Worksheet Use this one-page worksheet to track incidents and identify your child's bite type. Write down what was happening before the bite, where, with whom, and what your child's emotional state was. After three to five incidents, a pattern will emerge.

Tool 2: Signal Teaching Tracker Use this tracker to mark which of the five signals your child is learning each week. One signal per week is the right pace. Mark the date your child first signs independently.

Your Bonus: Included

Both tools are included with this guide. You have everything you need to get started today.

The Toddler Biting at Daycare Guide — available in your Lemon Squeezy library

Page 10: Lemon Squeezy — Ready to Go Deeper?

If this guide was helpful, there is more.

For New Parents: If you purchased this guide directly and want access to the full library of parent guides — covering sleep, mealtime battles, speech development, and more — use the link below to create your account and access every guide we've built.

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This guide is for informational purposes only and does not constitute medical or psychological advice. If your child bites frequently, causes injury, or if you are concerned about developmental delays, consult your pediatrician, a speech-language pathologist, or a licensed early childhood mental health professional.

If you are in the United States and your child is under 3, contact your state's Early Intervention program (Part C IDEA) for a free developmental screening. If your child is 3 or older, contact your local school district's preschool special education office.